PRESCRIPTION DRUG LAWS: JUSTIFIED HARD PATERNALISM

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The moral status of prescription drug laws has given philosophers pause. It is not that most philosophers think that these laws are morally unjustified. But the ground of their justification is not immediately obvious because prescription drug laws seem paternalistic, and many think that paternalism is a suspect ground of criminal legislation. Many authors have distinguished hard from soft paternalism and argued that the former is never permissible. They then argue that prescription drug laws are examples of soft, not hard, paternalism and are therefore morally legitimate.[1] Let us call these authors 'soft paternalists'. I argue that prescription drug laws are instances of permissible hard paternalism.

Defenders of hard paternalism are few and far between. H. L. A. Hart and Gerald Dworkin have toyed with the idea. John Kleinig has offered an extended defence of hard paternalism.[2] I consider and reject Kleinig's view in section two. Section one contains some introductory remarks. In section three, I argue that there is no soft paternalistic justification of prescription drug laws. In section four, I consider other examples of permissible hard paternalism as well as two other strategies for determining the limits of paternalism. I conclude that there are instances of permissible hard paternalism and that the only acceptable strategy is to balance harm and benefit on a case-by-case basis.

[1] The list of authors who have made this argument is long. It includes: Richard J. Arneson, 'Mill versus Paternalism,' *Ethics*, vol. 90, no. 4 (July 1980), pp. 470–489. Michael D. Bayles, *Principles of Legislation* (Detroit: Wayne State University Press, 1978). Joel Feinberg, *Harm to Self* (New York: Oxford University Press, 1986). John D. Hodson, 'The Principle of Paternalism,' *American Philosophical Quarterly*, vol. 14, no. 1 (January 1977), pp. 61–69. Donald VanDeVeer, *Paternalist Intervention* (Princeton: Princeton University Press, 1986).

[2] Gerald Dworkin, 'Paternalism: Some Second Thoughts,' in Rolf Sartorius *Paternalism* (Mineapolis: University of Minnesota Press, 1983), pp. 105–112. H. L. A. Hart *Law, Liberty and Morality* (Stanford: Stanford University Press, 1962) p. 33. John Kleinig *Paternalism* (Totowa, NJ: Rowman and Allanheld, 1984).

I

Paternalism is, roughly, interference with someone's liberty for their own good.[3] Paternalistic action is *soft* (or weak) when the cognitive or affective state of the individual on whose behalf an action is performed is significantly defective in a manner which interferes with decision-making. Authors have designated this cognitive or affective defect 'impairment', 'encumbrance', 'incompetence' or 'involuntariness'. If we have reason to think that a person is incompetent, or his decision is impaired, encumbered, or involuntary, then interference with his liberty on his behalf is an example of soft paternalism.[4] If, on the other hand, we have no such reason, then interference on his behalf is an example of *hard* (or strong) paternalism.

For our purposes we need not consider the difference between the terms 'impaired', 'incompetent', 'encumbered' and involuntary'. The idea behind them all is the same. For convenience I adopt the term 'involuntary'. A person's decision is involuntary when he: (1) lacks relevant knowledge, (2) is physically or mentally coerced, (3) is under the influence of distracting emotions or pain, (4) has defective reasoning faculties. Any one of these four factors can make a decision involuntary. The decision to cross a certain bridge is involuntary if the person who wishes to cross does not know that the bridge is unsafe.[5] His decision is involuntary because he suffers from a cognitive defect—his ignorance about the safety of the bridge. Action to prevent him from crossing the bridge is an example of soft paternalism.

Voluntariness is a decision-relative, threshold concept.[6] It is decision-relative because important or complex decisions will require a higher standard of voluntariness than less important or less complex decisions. Suppose Tom is a six-year-old. His decision to have chocolate ice cream, rather than vanilla, is voluntary. But Tom's decision to invest a large amount of money in a certain way would not be voluntary. So I speak of a decision

[3] 'Interference' must be taken rather broadly. For instance one can interfere by withholding information. See Allen Buchanan, 'The limits of Proxy Decision-Making,' in Sartorius, pp. 153–170.

[4] Interference is also soft if one interferes to determine the cognitive state of an actor. We can ignore this complication by assuming that we have certain knowledge of the cognitive states of the people in my examples.

[5] Example from John Stuart Mill, *On Liberty* (Indianapolis: Bobbs-Merrill, 1956).

[6] This discussion of voluntariness draws primarily on Joel Feinberg's explication of the concept. See pp. 98–123.

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being 'sufficiently voluntary': Tom's cognitive state is such that his ice cream decision is sufficiently voluntary but his investment decision is not. Voluntariness is a threshold concept because, once we determine the level of voluntariness required, the decision of anyone whose cognitive apparatus has that level of perfection *or better* is sufficiently voluntary. Tom's and Albert Einstein's decisions concerning ice cream flavors are both voluntary. Albert has a much more developed cognitive apparatus so his investment decisions are also voluntary. Relative to the ice cream decision, Albert's extra cognitive powers are an unused surplus.

The soft paternalist argues that one ought not to interfere with sufficiently voluntary decisions. She does *not* hold that the mere presence of any one of the four voluntariness-reducing factors legitimates paternalistic interference. Tom is an example of this. He is of course ignorant about many features of ice cream (that it contains fats, that it is made from milk, etc.). Nevertheless, given the simplicity and triviality of the decision, his choice is sufficiently voluntary and therefore we ought not to interfere with it.

The debate concerning the justification of prescription drug laws takes place within a larger debate over the justification of criminal legislation in general. Philosophers have distinguished four principles which might justify such legislation. Here is a rough statement of these principles:

(1) The harm-to-others principle: that an action tends to cause harm to an individual other than the actor is a good reason for criminal legislation.
(2) Legal paternalism: that an action tends to cause harm to the actor is a good reason for criminal legislation.
(3) The offence principle: that an action tends to offend people is a good reason for criminal legislation.
(4) Legal moralism: that an action is immoral is a good reason for criminal legislation.

In this paper I consider legal paternalism and the harm-to-others principle. The offence principle is not relevant and I assume that we will have no truck with legal moralism.

Note that hard paternalism in other contexts (e.g. between spouses) would seem to be more plausible than hard paternalistic action by the state. Laws must be general and the state has limited knowledge of special circumstances. Therefore, it will be more difficult to justify paternalistic action by the state. If hard *legal* paternalism is justified, then hard paternalism is surely justified in other contexts. I examine the arena in which the soft paternalist can make her best case.

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II

John Kleinig has provided an extended defence of hard paternalism.[7] His view deserves special consideration.

> Where our conduct or choices place our more permanent, stable, and central projects in jeopardy, and where what comes to expression in this conduct or these choices manifest aspects of our personality that do not rank highly in our constellation of desires, dispositions, etc., benevolent interference will constitute no violation of integrity. (. . .) Strong [hard], no less than weak [soft], paternalism may thus find a toe hold.[8]

Kleinig notes that some of our desires and projects are more central to our personalities than others. He argues that if a person chooses to forward one of her less central desires to the endangerment of one of her more central desires (and certain other conditions are met), then we may intervene even if the choice is voluntary.

There is an obvious objection to this line of thought. If one chooses a less central desire over a more central one, then either (1) the chosen desire is not really less central or (2) the choice is not voluntary. Kleinig considers this objection. He argues that it turns on a confusion between two senses of voluntary—the threshold and the absolute. An act is voluntary in the absolute sense when there are absolutely no voluntariness-reducing factors present. Absolute or full voluntariness is rarely, if ever, attained. An act is voluntary in the threshold sense when the actor's choices are sufficiently voluntary.

> Decisions or acts will need to be very substantially less than fully voluntary before they fall below the threshold of voluntariness.[9]

Kleinig argues for the threshold conception of voluntariness because he feels that the standard or full voluntariness would allow too much paternalism. So he reasons that one can make a mistake about the ordering of one's desires and still meet the threshold necessary to make an act voluntary. When this occurs and other conditions are met (e.g. the harm is great, enforcement costs are low), then hard paternalism is permissible.

[7] The critique of Kleinig presented in this section also applies to the defence of hard paternalism offered by Robert Young, 'Autonomy and Paternalism,' *Canadian Journal of Philosophy*, Supplement no. 8 (1982) pp. 47–66.

[8] Kleinig, p. 68.

[9] Kleinig, p. 69.

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Kleinig has failed to see that the clever soft paternalist will adopt the decision-relative conception of voluntariness outlined in section I. He speaks of one threshold of voluntariness—*the* threshold necessary for general competence. If there is only one threshold, then a decision could be important and one could meet the threshold and yet make a serious mistake which would require paternalistic intervention. But the soft paternalist will argue that the voluntariness threshold is decision-relative and so this situation cannot occur. She will argue that if a very serious mistake is possible, then the threshold must be raised high enough to ensure that no such mistakes are possible (or at least high enough to make them unlikely). The soft paternalist will respond to Kleinig's mistaken desire ordering as follows: either the mistake is not a serious one and so paternalistic intervention is not necessary, or the mistake is a serious one, in which case the threshold of voluntariness will rise and our intervention will not be an instance of hard paternalism.

If the soft paternalist's response is to succeed, she must provide a case-by-case analysis of seemingly paternalistic intervention. In each case, she must demonstrate that her strategy succeeds—that either the mistake does not justify interference or that interference is not hard paternalistic (because the threshold of voluntariness rises). If we can find one case where her strategy fails, then we have refuted the soft paternalist. If we can find a serious mistake which the threshold of voluntariness cannot rise to cover, then we will have a case of permissible hard paternalism. The next section presents such a case: prescription drug laws.

III

In the United States before 1938 one did not need a prescription to buy drugs. The sale of some drugs was illegal. But one could buy any legal drug without a prescription. Under the current law the sale (but, oddly, not the possession) of certain drugs without a prescription is illegal.[10] These laws are certainly paternalistic. Their effect is to require, not a note from your mother, but a 'note' from your doctor to obtain certain drugs.

The soft paternalist seeks a justification for these laws consistent with her rejection of hard paternalism. She has two possible approaches. She might argue that, given the serious

[10] James B. Bakalar and Lester Grinspoon, 'Drug Abuse and Social Attitudes to Risk Taking,' in Thomas H. Murray's *Feeling Good and Doing Better* (Clifton, NJ: Humana Press, 1984) pp. 13–26.

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consequences at stake, the decisions concerning these drugs are typically insufficiently voluntary. Or she might argue that these laws are justified on grounds that they are necessary to prevent harm to others. I consider each approach in turn.

Some authors argue that prescription drug laws are not examples of hard paternalism because people, in general, do not know very much about drugs.[11] As Mill's unsafe bridge example illustrates, ignorance reduces voluntariness. The soft paternalist might argue that because people do not know much about drugs their decisions are not sufficiently voluntary and, therefore, the state's interference with these decisions is not an instance of hard paternalism.

Let me quickly outline my response before turning to a more thorough presentation. While people do not know much about drugs, they do know that they are ignorant. People are perfectly aware (1) that drugs can be harmful if misused, and (2) that they do not know much about drugs. Suppose Jane, a typical layman, knows these facts and decides to buy and take a certain drug without seeing a doctor. She has done this with her 'eyes wide open.' If there is no special reason to think that Jane's choices are involuntary (e.g. no coercive threats or offers, no neurotic state), then Jane's choice to buy and take these drugs is sufficiently voluntary and any interference is an example of hard paternalism.

This response to the soft paternalist turns on the distinction between first-level knowledge and metaknowledge. First-level knowledge is knowledge of specific facts within a certain domain. Metaknowledge is knowledge of the extent of one's first-level knowledge. Suppose Jane happens to be a philosophy graduate student who specializes in moral philosophy. She has some first-level knowledge of logic but much less than she has of moral theory. But she has a great deal of logical metaknowledge. She usually recognizes when a philosophical question raises logical issues which she cannot deal with. She seeks out other philosophers to help her consider these issues. Jane has metaknowledge of more mundane matters. She does not have much first-level knowledge about cars, but she has good metaknowledge. She is aware of the limits of her knowledge of cars and that she can consult mechanics at appropriate times (e.g. every 3,000 miles or when her car makes odd noises).

The relationship between first-level and metaknowledge is complex. A fully voluntary choice would be made with full first-level knowledge and full metaknowledge. One can imagine the

[11] For example, see Feinberg, pp. 127–133.

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possible extremes. A person could have vast first-level knowledge but labour under the false conviction that he was omniscient and so have little metaknowledge. Socrates purportedly was the only man who knew that he knew nothing. Taken literally, this is a claim to have no first-level knowledge but at least some metaknowledge. In general it seems that the two types of knowledge are loosely related. An increase in first-level knowledge *usually* implies an increase in metaknowledge and vice versa. A detailed discussion of the interaction between first-level and metaknowledge is not necessary for our purposes. The distinction is clear enough for us to proceed.

Jane, like most of us, has very little first-level knowledge of medicine. She does not know much about drugs and their effects. But she has very accurate metaknowledge. She knows the extent of ignorance and that there are doctors whom she can consult at the appropriate times (e.g. every year or when her body does odd things). If metaknowledge makes choices concerning drugs sufficiently voluntary (and no other defence of drug laws succeeds), then prescription drug laws are examples of hard paternalism. But if first-level knowledge is required to make these decisions sufficiently voluntary, then these laws are not instances of hard paternalism.

In this debate, Joel Feinberg is the only author to consider the distinction between first-level and metaknowledge. In *Harm to Self* he considers the case of a sportsman who is planning to go ice-boating on a frozen lake. The sportsman does not know anything about the ice on this particular lake. He does have some general background knowledge. He knows that it is early April, that thaws can begin at this time, and that there is a chance that the ice is too thin to support his weight. He does not know about the ice on this lake, about the average temperature in this area and many other relevant facts.

> He knows that he doesn't know these things and that they are important. Ice-boating would be fun . . ., so knowing the risk (including that part of the risk that consists of his own ignorance), he proceeds.[12]

In our terms, the sportsman has little first-level knowledge but a great deal of metaknowledge. Feinberg concludes, with some hesitation, that:

> . . .on the whole one is inclined to say that since the sportsman

[12] Feinberg, p. 161.

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> understood exactly what risk he was taking, his taking it was voluntary enough.[13]

So Feinberg thinks that one ought not interfere with the sportsman's decision.

The case of the sportsman is exactly analogous to the epistemic position of most of us concerning drugs. We know that we do not know certain things about drugs and that the things we do not know are important. Feinberg would seem constrained to hold that prescription drug laws are instances of hard paternalism. As Feinberg is a soft paternalist, he would seem obliged to hold that these laws are unjustified.

However, at another point in *Harm to Self*, Feinberg explicitly states that drug laws are legitimate because people lack the relevant first-level knowledge.

> As a general rule, if a layman disagrees with a physician on a question of medical fact, the layman may safely be presumed wrong . . . Hence the state intervenes to protect him not from his own free choices, but from his factual ignorance.[14]

Given the hesitation with which Feinberg puts forth his conclusion in the sportsman case, we might assume that he would retract it in the face of its application to the drug case.

Beyond Feinberg's response, a broader question is raised. Can a consistent soft paternalist argue that metaknowledge is generally insufficient to make important decisions voluntary? I do not think she can. First of all, Feinberg's analysis of the sportsman case matches our considered moral judgments. More importantly, requiring first-level knowledge to make important decisions sufficiently voluntary would permit a great deal of liberty interference. Requiring first-level knowledge would imply that if a person lacks first-level knowledge of an important matter, then there is a reason for liberty interference. Suppose that Fred is considering whether or not to repair the brakes on his car himself. Fred has only a seventh-grade-shop knowledge of cars and fully realizes his ignorance. But he has been annoyed by the high prices and disdainful attitude of trained mechanics. Fred realizes the danger of repairing his own brakes. Fred will drive his car on his private racetrack so the harm-to-others principle is irrelevant. If the soft paternalist required first-level knowledge to make important decisions voluntary then she would have to admit that

[13] Feinberg, p. 161.
[14] Feinberg, p. 128.

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a law forbidding people in Fred's situation from doing their own car repairs is permissible.

Cases like Fred's are not rare. In our complex world we have good first-level knowledge only within the small area in which we have chosen to specialize. Most of us have very little first-level knowledge of cars, taxes, house construction, plumbing, etc. We frequently know only when to consult mechanics, tax preparers, building contractors, plumbers, etc.

The soft paternalist might argue that these examples are not as important as drug laws and so the threshold of voluntariness is higher in the drug case than it is in the cases just mentioned. Elsewhere I have argued that medical decisions are not significantly more important than many other kinds of decisions.[15] The sportsman is obviously risking his life. The proper maintenance of one's car is also a life-risking decision. For some individuals taxes involve huge sums of money. Moreover, many drug decisions are not matters of life and death. The consistent soft paternalist who required first-level knowledge would not be much of an anti-paternalist. She would think that paternalistic interference is permissible in many cases.

The ability to decide when one knows enough is crucial to the personal autonomy that the soft paternalist is determined to protect. The power to decide when, and when not, to seek information is a central part of what it means to be autonomous. For this reason, someone who wishes to argue that autonomy has some privileged moral status must hold that good metaknowledge makes even important decisions sufficiently voluntary. The soft paternalist cannot argue that prescription drug laws are legitimate because people lack first-level knowledge.

But there are other avenues for the soft paternalist defender of prescription drug laws who wishes to show that decisions concerning drugs are typically insufficiently voluntary. People decide whether or not to take drugs when they are ill. Illness can make reasoning more difficult. The soft paternalist might argue that this, not the lack of first-level knowledge, is what makes decisions concerning drugs insufficiently voluntary.

The extent to which illness reduces voluntariness has been exaggerated. Recalling the people one has seen buying prescription drugs, one will notice that they are not usually debilitated with pain or deluded because of their illness. Beyond this simple observation, note that soft paternalists are often defenders of

[15] George W. Rainbolt, 'Competition and the Patient-Centered Ethic,' *The Journal of Medicine and Philosophy*, vol. 12 (1987) pp. 85–99.

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consent forms in medical contexts. They think that patients with serious illnesses which require hospitalization retain enough of their mental faculties to make important and complex choices.

Suppose we set aside these problems. The soft paternalist defender of drug laws faces a more serious one. She must support the least restrictive method which ensures that people's choices are not unduly influenced by illness. Prescription drug laws are manifestly not the least restrictive way to achieve this objective. If we were to take this worry seriously, we would not require people to see physicians in order to obtain prescriptions. We would put large red labels on certain drugs which read: 'WARNING—The Food and Drug Administration has determined that this drug has potentially serious side-effects.' We would run large ad campaigns to reinforce these labels and teach children that taking drugs without a physician's advice was very risky. We would require pharmacists to ask every purchaser whom they suspected of being under duress several questions to make sure that they fully understood the risks they were taking. Given the risks involved, we would require pharmacists to refuse to sell drugs to anyone who showed any evidence (e.g. wincing in pain, sweating) or being too distracted by their illness to purchase drugs voluntarily. The institutional changes required would obviously be great. But the consistent soft paternalist must favour these kinds of safeguards over prescription laws.

I have now established that the soft paternalist defender of prescription drug laws has failed to show that drug decisions are insufficiently voluntary. I turn to possible justifications of these laws on the grounds that they prevent harm to others. It has been proposed that these laws are designed to protect society from the expense of drug abuse. If drugs were available without prescription some people would abuse them, and these people would then use hospitals, ambulances and other health care resources which are already in short supply. This extra use would force the rest of us to pay higher taxes to support public health care services. In this way the absence of prescription laws would cause harm to individuals besides the purchaser of the drug and therefore we may rightly require prescription on harm-to-others grounds.

While examining the legitimacy of laws requiring motorcyclists to wear helmets, Gerald Dworkin considers this line of thought.[16] He points out that a mandatory insurance scheme would ensure that all costs were borne by the individuals who purchased the drugs. Under this system, if one wished to buy drugs without a

[16] Dworkin, p. 108–110.

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prescription, one would have to show proof of insurance. This would eliminate these costs without resorting to prescription laws.

I see no other plausible reason to suppose that prescription drug laws are not examples of hard paternalism. At this point the soft paternalist defender of drug laws is faced with a dilemma. She can (1) abandon soft paternalism, or (2) abandon her defence of these laws. At least two authors have chosen the second horn of this dilemma. John Stuart Mill considers the sale of poisons and argues that it would be illegitimate 'to require in all cases the certificate of a medical practitioner . . .'[17] He argues for warning labels and questions by pharmacists such as those discussed above. In discussing an article by Feinberg, Richard J. Arneson agrees with Mill:

> Unfortunately, Feinberg sets the requirements for fully voluntary choice so high that his use of the voluntariness standard . . . results in the endorsement of state regulation of the traffic in medical drugs . . .[18]

Arneson assumes that Mill was correct in rejecting prescription laws.

In grasping the second horn of the dilemma, Arneson and Mill might argue that if we (1) required insurance, (2) put on warning labels, and (3) trained our pharmacists to spot people who were unable to make sufficiently voluntary decisions, then prescription laws would no longer be necessary. But even if we made these restrictions, people would still make serious, although sufficiently voluntary, mistakes. They would take drugs which aggravated their illnesses. People would make mistakes which cost them their lives. Arneson thinks that we must live with this state of affairs. He thinks that one's decision not to see a physician is sufficiently voluntary and so we cannot interfere. Therefore, we must stand by and let people do serious harm to themselves.

This is a consistent soft-paternalist position. In response to any proposed case of permissible hard paternalism the soft paternalist can always preserve consistency by claiming that the mistakes involved are not serious enough to warrant liberty interference. Arguing against this sort of move is difficult. I can only ask Arneson and Mill to vividly imagine the consequenes of repealing prescription drug laws. If they remain unconvinced then all I have established is a conditional: *if* prescription drug laws are justified, *then* they are justified on hard paternalistic grounds.

[17] Mill. p. 117.
[18] Arneson, p. 482.

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However, I think that if one considers the consequences of the repeal of prescription drug laws noted above, one will accept the antecedent of the conditional. Even autonomy is not worth this price. We ought to grasp the first horn of the dilemma. The consequences of the second are so awful that we ought to accept hard paternalism.

IV

This conclusion is reinforced if one considers some of the other hard cases for the soft paternalist. Take, for example, laws requiring people to wear a helmet when riding a motorcyle or a seat belt when driving. The issues surrounding the legitimacy of these laws are much the same as those we have been considering. In defending the soft paternalist line, authors have been driven to positions which seem very *ad hoc*. Some argue that these laws are legitimate on harm-to-others grounds because of the psychic costs incurred by people who hit unhelmeted cyclists and unbelted motorists as well as the psychic cost incurred by people who see the results of such accidents.[19]

This line of thought would license all sorts of odd interferences with liberty. Suppose Bob and Sue are at the beach and Sue wants to swim. Bob cannot swim and there happens to be no one else on the beach. If Sue drowns, Bob, watching helpless on the beach, will suffer great psychic costs. But obviously these costs do not legitimate Bob's interference with Sue's liberty to swim. Others have argued that helmet and seat-belt laws are justified because of the strain these injured people put on the health care system.[20] But, as we noted above, this defence supports only mandatory insurance laws.

Acknowledging that hard paternalism is sometimes permissible would allow us to give the most obvious and natural explanation of these laws—they are cases of permissible hard paternalism. In these cases the harm prevented is great, the liberty interference is trivial and the enforcement costs are low. This constellation of facts justifies hard paternalism. Thus we give a very natural justification of helmet and seat-belt laws. But the soft paternalist is worried that once the absolute prohibition of hard paternalism is abandoned, there will be no way to check state intrusions into our liberty. Obviously paternalism must be limited in some way.

Let us consider briefly two other strategies for determining the

[19] Dworkin, p. 110 and Feinberg, p. 140-141.
[20] VanDeVeer, p. 306

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limits of paternalism: hypothetical rational consent and subsequent consent. Dworkin and John Rawls have suggested that paternalism is permissible if a fully informed, fully rational agent would consent to the interference.[21] The difficulty with this view is that it licenses too much paternalism. The phrase 'fully rational' needs to be explicated. For example, one needs to know what risk attitude the fully rational agent will adopt. Will he be risk-averse, risk-neutral or risk-positive? Suppose we decide the fully rational agent is risk-neutral. Consider those who refuse to ride in cars because they think it is too dangerous. They are risk-negative. It is unjustifiedly paternalistic to impose the choices of a fully rational, risk-neutral agent on them. In general, we must define the character of the fully rational agent if this attempt to determine the limits of paternalism is to give us determinate answers. Any definition of 'fully rational' would impose values on those who do not fit the definition.[22]

Another possibility is subsequent consent. On this view paternalism is permissible if and only if the agent would consent to it at a later date.[23] This view begins from the observation that a common justification of paternalistic action towards children is that at a later date they will be glad that the action was taken. Billy does not want to eat his vegetables, but, years later, William is glad his parents taught him good eating habits.

This view has little plausibility. First of all, suppose Billy died before he grew up. Then he did not give subsequent consent and the paternalism was unjustified. This is a very odd implication. Moreover, the paternalistic act of one time may bring about consent at a later date. Consider an extreme case. Suppose a normal person were given a psychiatric treatment which she did not consent to. Part of the treatment makes the patient change her values and she later consents to the treatment. Manipulation of the grossest sort would be justified by this theory.

V

I have argued that the hard/soft distinction does not serve to establish the limits of paternalism and that no 'principle of

[21] Dworkin, p. 107 and John Rawls, *A Theory of Justice* (Cambridge: Belknap Press, 1971) pp. 209.

[22] There is a tendency for defenders of this view to move from 'fully informed, fully rational agent' to 'fully informed, actual agent.' However, this move makes them soft paternalists and subject to the argument presented in the last section.

[23] I can find no one who actually adopts this view. It has been considered by several of the authors listed above. See for example VanDeVeer, pp. 66–70.

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paternalism' exists which accords with our considered moral judgments. We must treat paternalism as we treat the harm-to-others principle. In both cases the search for a simple principle is fruitless. We must adopt a balancing strategy— balancing, among other things, autonomy and well-being. We must be content with some rules of thumb which provide guidelines, but not hard and fast rules. An adequate account of these rules of thumb would require another paper. But I end with some possible rules. The list is intended to be suggestive—not exhaustive or adequately explicated.

(1) Paternalism is a good reason for liberty interference. It is not necessarily sufficient.
(2) Paternalism has a heavy cost in autonomy. Therefore we must know:
    (a) that the benefits outweigh these costs, and
    (b) that there is no other less costly way to gain the benefits.
(3) Some infringements on autonomy are more serious than others. Paternalism is more likely to be acceptable when only a minor infringement is required.
(4) The costs of enforcing paternalism must be low.

Prescription drug laws, as well as helmet and seat-belt laws, are instances where the costs of paternalism are outweighed by the benefits. In these cases the liberty infringement is minor, enforcement costs are low, the harm avoided is great, and there is no other practical way to avoid the harm.

The fear of hard paternalism has been overstated. It is said that allowing hard paternalism to be a reason for legislation would 'justify the imposition of a Spartan-like regime requiring rigorous physical exercise and abstention from smoking, drinking and hazardous pastimes.'[24] If one considers the guidelines just mentioned, it becomes apparent that such a fear is unjustified.[25]

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[24] Robert Harris, 'Private Consensual Adult Behavior: The Requirement of Harm to Others in the Enforcement of Morality,' *U.C.L.A. Law Review*, vol. 14 (1967) p. 585n.

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